ConvergeFlow
From appointments to verified coverage

DENTAL CODING & BILLING

Dental billing, fully managed. From the chair to a closed account.

ConvergeFlow handles verification, CDT coding, claims, payments and follow-up for US dental practices, so your team can focus on patients.



Benefits verified
before treatment begins

CDT-accurate claims
matched to the clinical record

Balances resolved
with a documented owner

We don't just report the problem. We fix it.

Dental claims fail for dental reasons: a missing tooth number, an expired frequency limit, a radiograph that never got attached. Our team works the whole cycle so those gaps get caught before the payer finds them.

- **Fully managed service**
  Our specialists own the work, not just a dashboard.
- **Built into your PMS**
  Starting with OpenDental, results land where staff already look.
- **Dental-specific expertise**
  CDT codes, tooth and surface detail, narratives and attachments.
- **Clear accountability**
  Every handoff has a named owner and a documented status.
- **Evidence for every step**
  Payer references, dates and submission proof retained.
- **Reporting you can act on**
  Daily queues and weekly trends, on measures we agree together.

THE DENTAL REVENUE CYCLE

Four stages, one accountable team

01
Before treatment
Registration, benefits verification, treatment planning and estimates.

02
Clinical & claim prep
Documented care, validated CDT codes and claim attachments.

03
After submission
Payer acceptance, payment posting, denials and secondary claims.

04
Account closure
Verified patient balances, reconciled credits and reporting.

The cycle ends only when the account has a documented financial resolution.

01

BEFORE TREATMENT

Clean claims start at the front desk.

Registration & scheduling
Legal name, date of birth, subscriber relationship and member ID verified; duplicate records caught.

Provider & plan
Treating dentist, service location, plan type and network participation confirmed.

Treatment planning & estimates
Expected fees, contract allowances and verified benefits applied, so patients understand their share before care.



Dental benefits verification

Active coverage is only the starting point. We check the details that actually decide payment, and record the payer reference, date and source.

- Effective dates
- Deductible
- Covered categories
- Patient share
- Annual maximum remaining
- Waiting periods
- Age limits
- Exclusions
- Frequency limits
- Replacement limits
- Prior service dates
- Missing-tooth provisions

Predetermination
Advance benefit estimates requested for proposed treatment when the plan offers them.

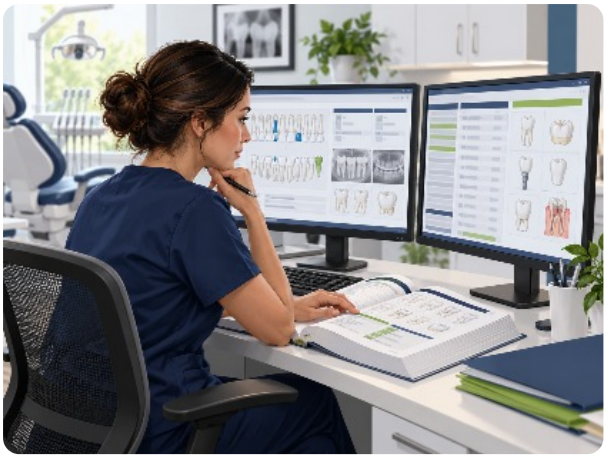
Prior authorization
Approvals obtained where plans require them, with decision, reference and valid dates tracked.

Benefits vary by plan. We recheck coverage when treatment or the service date changes. An estimate helps the patient plan; it does not guarantee the final insurance payment.

02

CLINICAL & CLAIM PREPARATION

Every claim matches the chart.



Clinical documentation support

Findings, tooth or area, surfaces, materials and supporting radiographs complete and signed before billing.

CDT coding

The CDT edition in effect for the date of service, matched to the care actually documented.

Charge capture

Completed appointments reconciled against the ledger, with multi-visit procedures billed on the right date.

Anatomy that matches the procedure

ADA Universal tooth numbers (1–32, A–T), treated surfaces (M, D, O, I, B, L) and quadrant or arch, whenever the code requires them.

EXAMPLE CHECK

A restoration on tooth 30 with mesial and occlusal surfaces must match what the clinical record shows.

Attachments & narratives

Legible, correctly labelled images and a clear narrative: tooth, findings, reason for care and history.

Pre-submission review

Code version, anatomy, duplicates, attachments and authorizations validated; every hold assigned and cleared.

Claim creation

Subscriber, group, provider NPI, tax ID and procedure lines checked against reliable sources.

Electronic submission

837D claims sent through approved channels, with batch, claim ID and attachment linkage retained.

When a record is unclear, we ask the dentist. Reimbursement preferences never determine the code.

03

AFTER SUBMISSION & CLOSURE

Paid accurately. Closed cleanly.

Acknowledgments & rejections
Clearinghouse and payer reports reviewed; errors fixed at the source and resubmitted.

Payer decisions
Deductibles, limits, exclusions and alternate-benefit allowances reviewed against your contract.

Payment posting
ERAs and EOBs posted to the right claim and matched to EFT or check receipts.

Secondary coverage & COB
Payer order confirmed and primary results passed on before the final statement.

Denial & underpayment review
Root cause classified, then corrected, documented, reconsidered or appealed on deadline.

A/R & patient billing
Balances worked by age and value; patients billed only for verified amounts.

How a dental claim balances

Illustrative example

$1,000 charge · $800 contract allowance · deductible met · 50% plan benefit



$400 + $400 + $200 = $1,000. Real liability depends on the plan, contract and applicable rules. A contractual adjustment is never billed to the patient.

SPECIALTY WORKFLOWS

Orthodontics
Lifetime limits, age rules, treatment dates and installment billing.

Periodontics & prosthodontics
Clinical evidence, history and replacement information.

Medical crossover
Cases that may qualify under medical benefits, handled with their own claim rules.



An operating model you can see.

Before we start, we agree on scope, owners and how we'll report, so you always know where every claim stands.

1

Practice inputs
Complete clinical records, provider details, payer contracts and timely clarification.

2

Billing workflow
Agreed coding review, claim preparation, submission monitoring, posting and follow-up.

3

Access setup
Approved system access, payer connectivity, secure exchange and privacy arrangements.

4

Launch review
Work queues, escalation contacts, reporting cadence and acceptance criteria confirmed.

QUALITY CONTROLS & CLIENT REPORTING

Daily
Incomplete records, claims on hold, rejections and unposted receipts.

Weekly
Denial causes, aging balances, underpayments and attachment delays.

Defined measures
Charge lag, claim acceptance, denial rate and collections, defined with you.

LET'S TALK

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Targets are set from your baseline and agreed scope. No reimbursement outcome is guaranteed.